

✓ 19. A 40-year-old, female, bird breeder, presented with progressive dyspnea and cough. Her chest X-ray detected pulmonary infiltrates and her spirometry tests revealed low Vital capacity and FVC while FEV1 and FEV1/FVC. The most probable diagnosis is *1/1

- ☒ A. Interstitial lung disease ✓
- ☐ B. Bronchial asthma
- ☐ C. COPD
- ☐ D. Pneumonia

✓ 20. Spirometry cannot be used to * 1/1

- ☐ ● Differentiate obstructive from restrictive pulmonary disorders
- ☐ ● Characterize severity of airway obstruction
- ☐ ● Measure responses to therapy of obstructive airway disease.
- ☒ ● Diagnose pneumonia ✓

✗ 1. Pulmonary capillary wedge pressure (PCWP) in ARDS is: *0/1

- ☐ A. 20 mm Hg.
- ☐ B. < 18 mm Hg.
- ☒ C. 32 mm Hg. ✗
- ☐ D. 10 mm Hg.

Correct answer

- ☒ B. < 18 mm Hg.

✓ 2. Clinical features of ARDS * 1/1

- ☐ A. The acute inflammatory phase lasts 24 hours.
- ☒ B. The late fibroproliferative phase is associated with multi-organ failure. ✓
- ☐ C. Pneumothorax is common in the late fibroproliferative phase.
- ☐ D. The combination of cyanosis, dyspnea, confusion and lung crepitations is diagnostic.

✓ 3. What is the most appropriate place in therapy for long-acting beta-agonists? *1/1

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- ☐ A. As monotherapy.
- ☐ B. As rescue therapy.
- ☒ C. In combination with ICSs. ✓
- ☐ D. In combination with cromolyn.

✗ 4. Theophylline use is can be limited by which of the following? *0/1

- ☐ A. The need to constantly monitor serum levels.
- ☐ B. A narrow therapeutic index.
- ☐ C. Numerous drug and disease interactions.
- ☒ D. All of the above. ✗

Correct answer

- ☒ B. A narrow therapeutic index.

✓ 5. Which test is NOT used during diagnosis or monitoring of asthma? *1/1

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- ☒ A. Diffusing capacity. ✓
- ☐ B. Peak expiratory flow rate.
- ☐ C. Bronchial challenge test.
- ☐ D. Skin prick test.

✗ 6. What blood vessel carries deoxygenated blood from the heart to the lungs? *0/1

- ☐ A. Pulmonary vein
- ☐ B. Aorta
- ☒ C. Pulmonary artery ✗
- ☐ D. Vena cava

Correct answer

- ☒ D. Vena cava

✗ 7. Bronchial artery on the right side arise from _____. *0/1

- ☒ A. Descending aorta ✗

✗ 7. **Bronchial artery on the right side arise from _____.** *0/1

- ☒ A. Descending aorta ✗
- ☐ B. Carotid artery
- ☐ C. Subclavian artery
- ☐ D. Superior left bronchial artery

Correct answer

- ☒ C. Subclavian artery

✓ 8. **The cause of localized bronchiectasis is** *1/1

- ☐ A. Recurrent chest infection
- ☒ B. Foreign body aspiration ✓
- ☐ C. Cystic fibrosis
- ☐ D. Allergic bronchopulmonary aspergillosis

✗ **which of the following is true about surgical 9. management of bronchiectasis** *0/1

- ☐ A. It indicated in unilateral bronchiectasis

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- ☐ A. It indicated in unilateral bronchiectasis
- ☐ B. It indicated in central bronchiectasis
- ☒ C. It indicated in peripheral bronchiectasis ✗
- ☐ D. It indicated in localized bronchiectasis

Correct answer

- ☒ D. It indicated in localized bronchiectasis

✗ **10. Which of the following is not involved in interstitial lung diseases?** *0/1

- ☐ A. Capillary endothelium
- ☐ B. Perivascular tissues
- ☒ C. Lymphatic tissues ✗
- ☐ D. Pleura

Correct answer

- ☒ D. Pleura

✓ **11. Etiology of which of the** *1/1

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- ☒ C. It indicated in peripheral bronchiectasis ✗
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Correct answer

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- ☒ C. Lymphatic tissues ✗
- ☐ D. Pleura

Correct answer

- ☒ D. Pleura

✓ **11. Etiology of which of the** *1/1

✓ 11. Etiology of which of the following interstitial lung disease is known?

*1/1

- ☐ A. Sarcoidosis.
- ☐ B. Idiopathic pulmonary fibrosis (IPF).
- ☐ C. Pulmonary fibrosis associated with CTDs.
- ☒ D. Pneumoconiosis ✓

✗ 12. Concerning acute lung abscess, which of the following is true?

*0/1

- ☒ A. Less than two weeks. ✗
- ☐ B. Less than three weeks.
- ☐ C. Less than four weeks.
- ☐ D. Less than five weeks.

Correct answer

- ☒ C. Less than four weeks.

✓ 13. Which of the followings is useful to differentiate an abscess

*1/1

✓ 14. Which of the following spirometry results confirms severe COPD *1/1

- ☒ A. Post bronchodilator FEV1/FVC = 0.65, FEV1 45% ✓
- ☐ B. Post bronchodilator FEV1/FVC = 0.75, FEV1 45%
- ☐ C. Post bronchodilator FEV1/FVC = 0.75, FEV1 55%
- ☐ D. Post bronchodilator FEV1/FVC = 0.75, FEV1 80%

✓ 15. COPD makes it harder to breathe because *1/1

- ☐ A. The walls of the airways in the lungs become thick and inflamed.
- ☐ B. Bronchospasm
- ☐ C. The airways are clogged with excess mucus.
- ☐ D. The lungs lose their elasticity.
- ☒ E. All of the above. ✓

✓ 16 Siderosis is due to ? * 1/1

- ☒ A. Iron & iron oxides ✓
- ☐ B. Tin oxide
- ☐ C. Barium sulfate
- ☐ D. Antimony salts

✗ 12. Concerning acute lung abscess, which of the following is true? *0/1

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- ☐ B. Less than three weeks.
- ☐ C. Less than four weeks.
- ☐ D. Less than five weeks.

Correct answer

- ☒ C. Less than four weeks.

✓ 13. Which of the followings is useful to differentiate an abscess from a pleural collection? *1/1

- ☒ A. Computed tomography scan of chest. ✓
- ☐ B. Chest X-ray.
- ☐ C. PET scan.
- ☐ D. MRI.

✓ 14. Which of the following spirometry results confirms severe COPD *1/1

✗ 17. Inhaled corticosteroids for the long-term treatment of asthma exhibit the following desired feature: *0/1

- ☐ A-Enhance cellular response to inflammatory mediators.
- ☒ B-Increase production of leukotriene ✗
- ☐ C-Mast cell stabilization
- ☐ D- Relaxation of skeletal muscle

Correct answer

- ☒ C-Mast cell stabilization

✗ 18. Regarding to the drug interaction of theophylline. The half-life of theophylline is decrease by which of the following? *0/1

- ☒ A- Erythromycin ✗
- ☐ B- Phenobarbitone
- ☐ C- Quinolones
- ☐ D-Cimetidine

Correct answer

- ☒ B- Phenobarbitone

✓ -Organ most frequently affected in sarcoidosis is ? *1/1

☒ A. Lung ✓

☐ B. Skin

☐ C. Eye

☐ D. Lymph node.

✓ Löfgren's syndrome is characterized by ? *1/1

☐ A. Arthritis

☐ B. Erythema nodosum

☐ C. Bilateral hilar adenopathy

☒ D. All of the above ✓

✓ A 42 shepherd came to the primary care with chest pain and cough. What type of pneumonia does he have? *1/1

☒ A) CAP. ✓

☐ B) HAP.

☐ C) VAP.

✓ - Horner syndrome is due to compression on:

*1/1

☒ a. Sympathetic trunk ✓

☐ b. Left recurrent laryngeal nerve

☐ c. Esophagus

☐ d. Trachea

✓ - Compression on phrenic nerve lead to

*1/1

☐ a. Dysphagia

☐ b. Dysphonia

☐ c. Hoarseness of voice

☒ d. Diaphragmatic paralysis ✓

✗ . Which one of the following is not true regarding lung tumors? *0/1

- ☐ A. The most common benign lung tumors are pulmonary hamartomas, lung neurofibromas, or lipomas
- ☐ B. 70% of lung tumors are malignant
- ☐ C. Bronchogenic Carcinomas often present late when local invasion and metastases are already present
- ☒ D. 5- years survival is less than 15% ✗

Correct answer

- ☒ B. 70% of lung tumors are malignant

✗ . Which one of the following is not true regarding lung tumors? *0/1

- ☐ A. 97% of lung tumors are malignant.
- ☐ B. Bronchogenic carcinomas often present late when local invasion and metastases are already present.
- ☒ C. Lung cancer is by far the number one cancer killer. ✗
- ☐ D. 50% are due to smoking.

Correct answer

- ☒ D. 50% are due to smoking.

✓ . **Low Ventilation/Perfusion mismatch is:**

*1/1

- ☐ A. Ventilation without perfusion
- ☒ B. Perfusion without ventilation ✓
- ☐ C. Ventilation and perfusion are equal
- ☐ D. A cardiac output higher than minute ventilation

✓ . **Which of the following are not common causes of respiratory failure?**

*1/1

- ☐ A. Pneumonia
- ☐ B. Pulmonary embolism
- ☒ C. Pulmonary hemorrhage ✓
- ☐ D. ARDS

✓ . **Hypoxia brain effects depend on:**

*1/1

- ☒ A. Severity and duration of hypoxia. ✓
- ☐ B. Presence of cardiac or hepatic diseases.
- ☐ C. Anemia.

✓ . **Neurological and behavioral findings in patients with hypoxia included:** *1/1

- ☐ A. Somnolence.
- ☐ B. Tremor and asterix.
- ☒ C. Vertigo. ✓
- ☐ D. Memory impairment.

✓ **A combination of medications which are applied to treat tuberculosis is** *1/1

- ☐ A. To generate a better response.
- ☐ B. To decrease the resistance of the entity to the treatment.
- ☒ C. Both A and B. ✓
- ☐ D. None of these.

✓ **-This is the reason why diagnosing tuberculosis is turning challenging** *1/1

- ☒ A. Disease takes years to become active ✓
- ☐ B. Symptoms are irregular, they appear and then vanish



What is the most accurate enzyme to rule out TB in case of exudative lymphocytic pleural effusion?

***1/1**

- ☐ A. Amylase.
- ☒ B. Adenosine deaminase. 
- ☐ C. Pepsin.
- ☐ D. Lipase.



In bronchitis it is important to * **0/1**

- ☒ A- Sleep near a window 
- ☐ B- Drink plenty of fluids
- ☐ C- Eat less carbohydrates
- ☐ D- None of above

Correct answer

- ☒ D- None of above

✗ Which one of the followings lung cancer has a worse prognosis? *0/1

☒ A. Adenocarcinoma. ✗

☐ B. Bronchioalveolar carcinoma.

☐ C. Squamous cell carcinoma.

☐ D. Small cell carcinoma.

Correct answer

☒ D. Small cell carcinoma.

✗ . Which of the following is low flow oxygen delivery system? *0/1

☐ A. Partial rebreathing face masks with reservoir bags.

☐ B. Jet-Mixing Venturi Mask.

☐ C. CPAP.

☒ D. High flow nasal cannula ✗

Correct answer

☒ A. Partial rebreathing face masks with reservoir bags.

✓ A. 20) All the following are symptoms of CAP except: *1/1

✓ A. 20) All the following are symptoms of CAP except: *1/1

- ☐ F. A) Fever
- ☐ G. B) Dyspnea
- ☐ H. C) Cough
- ☒ I. D) Arthritis ✓

✓ All of the following can be used to diagnose atypical CAP EXCEPT? *1/1

- ☐ A) ESR and C-reactive protein.
- ☐ B) X-rays.
- ☒ C) Gram stain. ✓
- ☐ D) All the above apply

✗ Which of the following is the most accurate about the small pleural effusion not required thoracentesis *0/1

- ☐ A. Recent thoracic or abdominal surgery.
- ☐ B. Too small to safely aspirate.
- ☒ C. In clinically stable patients. ✗
- ☐ D. Presence can be explained by underlying congestive heart failure.

Correct answer

- ☒ B. Too small to safely aspirate.

1/1*

Diagnostic radiological .1 ✓
:criteria of ARDS is

A. Multiple cavitary lung lesions. ☐

B. Massive fibrosis. ☐

✓ C. Bilateral lung opacities that
progress into a more asymmetrical
shape. ☒

D. Pneumothorax. ☐

1/1*

In ARDS the exudative .2 ✓
:phase may last

A. 3-7 days. ☐



B. 7-10 days. ☒

C. 10-20 days. ☐

D. 30 days. ☐

1/1*

The cause of localized .3 ✓
:bronchiectasis is



1/1*

The cause of localized .3 ✓
:bronchiectasis is

A. Recurrent chest infection. ☐



B. Foreign body aspiration. ☒

C. Cystic fibrosis. ☐

D. Allergic bronchopulmonary aspergillosis. ☐

1/1* :Bronchiectasis is defined as .4 ✓



A. Abnormal permanent dilatation of bronchi. ☒

B. Abnormal permanent dilatation of bronchioles. ☐

C. Abnormal permanent dilatation of air space. ☐

D. Abnormal permanent dilatation of blood vessels. ☐

1/1*

A 55 years old female on .5 ✓
oral contraceptive pills,
presented to ER with sudden
shortness of breath and chest



1/1*

A 55 years old female on .5 ✓
oral contraceptive pills,
presented to ER with sudden
shortness of breath and chest
pain. On exam there is a wide
fixed splitting in second heart
sound with 3rd heart sound
heard along left sternal border.
The arterial blood gases findings
:in this case will showed

- A. Hypoxemia with hypercapnia. ☐
- B. Hypoxemia with normal A-a
gradient. ☐
- C. Hyperoxia with hypocapnia. ☐
- ✓ D. Hypoxemia with wide A-a
gradient. ☒

1/1*

Once pulmonary embolism .6 ✓
has been suspected, the next
:step is

- A. Heparin must be started. ☐
- ✓ B. Heparin must be started and
imaging studies must be requested ☒
- C. Prepare of surgical removal of
emboli. ☐
- D. Thrombolytics must be started. ☐

1/1* A 69 years old, recently .7 ✓
bedridden due to fracture femur.
He presented with shortness of
breath and fever. Which of the
following is highly sensitive for
predicting the absence of PE and
is used for patients whose
probability for PE is unlikely and
who may need only minimal
?additional testing

A. V\Q scanning. ☐



B. D- dimer testing. ☒

C. CT pulmonary angiography. ☐

D. Duplex ultrasonography. ☐

1/1* What drug therapy should be .8 ✓
available to all asthmatic
patients (intermittent and
?persistent)



A. Low dose inhaled
corticosteroids. ☒

B. Short acting beta agonist on
demand ☐

C. Long acting beta agonist. ☐

D. Leukotriene antagonist. ☐



D. Leukotriene antagonist. ☐

1/1* Theophylline use can be .9 ✓
limited by which of the
?following

A. Difficult route of administration. ☐

✓ B. A narrow therapeutic index. ☒

C. Numerous side effects. ☐

D. Expensive. ☐

0/1* A 30-year-old patient with a .10 ✗
history of mild persistent asthma
(baseline peak expiratory flow
rate of 85%) presents to the
emergency department with
shortness of breath and
wheezing that has not relieved
by her albuterol inhaler for the
past 12 hours. She was able to
tolerate pulmonary function tests
and a set was performed. Which
of the following is the most likely
?test result



0/1* A 30-year-old patient with a .10 history of mild persistent asthma (baseline peak expiratory flow rate of 85%) presents to the emergency department with shortness of breath and wheezing that has not relieved by her albuterol inhaler for the past 12 hours. She was able to tolerate pulmonary function tests and a set was performed. Which of the following is the most likely test result?

- A. Decreased FEV1, normal/increased FVC, decreased FEV1: FVC ratio, with post bronchodilator. ☐
- B. FEV1 increased by 13%. ☐
- C. Decreased residual volume and total lung capacity. ☐
- D. Increased FEV1 increased FVC, normal FEV1: FVC ratio. ☐
- ☒ E. Increased residual volume, increased total lung capacity, increased FEV1.

الإجابة الصحيحة

- A. Decreased FEV1, normal/increased FVC, decreased FEV1: FVC ratio, with post bronchodilator. ☒

1/1* Which one of the following .11 ✓
is risk for primary pneumothorax

A. Chest trauma. ☐

✓ B. Sub plural bleb or bullae. ☒

C. COPD. ☐

D. Asthma. ☐

1/1* A 60-year-old, male, Shisha .12 ✓
smoker, presented with cough
and toxic symptoms. Chest X-ray
showed upper right sided cavity
with air-fluid level, sputum
microbiological culture revealed
no growth. Patient started broad
spectrum antibiotic with no
improvement. One of possible
?causative organisms is

A. Staphylococcus aureus. ☐

B. Hemophilus influenza. ☐

C. Streptococcus pneumonia. ☐

✓ D. Mycobacterium tuberculosis
bacilli. ☒



1/1*

A 45-year-old, female, .13 ✓
presented with dyspnea and dry
cough. Chest X-ray showed left
sided cavity with air-fluid level,
patient started broad spectrum
antibiotics with no response for
?10 days. What is next step

✓ A. Bronchoscope for
microbiological analysis of
bronchoalveolar lavage. ☒

B. Referral to cardiothoracic surgeon. ☐

C. Thoracoscopic biopsy. ☐

D. Transthoracic percutaneous needle
aspiration (CT- or US-guided). ☐

1/1

* In tension Pneumothorax .14 ✓

A. Inter plural pressure equal to
atmospheric pressure. ☐

✓ B. Inter plural pressure more than
atmospheric pressure. ☒

C. Inter plural pressure less than
atmospheric pressure. ☐

D. No change in pleural pressure. ☐



1/1* Which one of the following .15 ✓
is true regarding bronchogenic
?carcinomas

A. Surgery is the most important treatment modality. ☐

B. Most patients of lung cancer are asymptomatic & presented with non-specific symptoms. ☐

C. Only 40% of lung tumors are malignant. ☐

✓ D. Para neoplastic syndrome is common association of bronchogenic carcinomas. ☒

1/1* Cavitation is common .16 ✓
:association with

A. Adenocarcinoma. ☐

B. Large cell carcinoma. ☐

C. Small cell lung cancer. ☐

✓ D. Squamous cell carcinoma. ☒



1/1*

Which diagnostic method .17 ✓
that differentiate between
mesothelioma and
?Adenocarcinoma

A. Pleural fluid chemical analysis. ☐

B. Computed tomography scan of
chest. ☐

C. Chest x-ray. ☐



D. Immunohistochemistry. ☒

1/1*

Stains that are usually .18 ✓
negative for mesothelioma
:include



A. CEA. ☒

B. Calretinin. ☐

C. Keratin. ☐

D. Podoplanin. ☐

0/1*

Which of the following .19 ✗
diseases increase tactile vocal
?fremitus

0/1*

Which of the following .19 ✕
diseases increase tactile vocal
?fremitus

A. Pneumonia. ☐

B. Asthma ☐

✕

C. Pulmonary fibrosis. ☒

D. Pneumothorax. ☐

الإجابة الصحيحة

A. Pneumonia. ☒

1/1*

Productive cough is mainly .20 ✓
:due to

✓

A. Airways diseases. ☒

B. Pleural diseases. ☐

C. Interstitial lung diseases. ☐

D. Pulmonary vascular diseases. ☐

0/1*

Potential clinical .21 ✖
:complication of silicosis is

A. Mycobacterium tuberculosis infection.

☐

. B. Rheumatoid arthritis.

☐

C. Scleroderma

☒

D. corepulmonale.

☐

الإجابة الصحيحة

A. Mycobacterium tuberculosis infection.

☒

1/1*

Coal workers .22 ✔
pneumoconiosis

A. Causes increased susceptibility to tuberculosis

☐

B. Of the complicated type is common and affects all coal workers after a period

☐

C. Causes coal nodules

☒

D. Predisposes the affected patient to lung cancer

☐

1/1*

Long acting beta two .23 ✓
agonist should be used with
inhaled corticosteroids due to
?which of the following

- A. Delay up regulation of receptors. ☐
- B. Enhance down regulation of receptors ☐
- ✓ C. Prevent occurrence of tolerance. ☒
- D. D- Decrease the hypokalemic effect. ☐

1/1

*

.....Stages of empyema are .24 ✓

- A. A-Primary and secondary ☐
- ✓ B. B- Exudative, fibro purulent and organized. ☒
- C. C-Empyema has no stages. ☐
- D. Early stage and late stage. ☐

1/1*

Which of the following types .25 ✓
of lung cancer causes
?gynaecomastia

1/1 *Stages of empyema are .24 ✓

A. A-Primary and secondary ☐

✓ B. B- Exudative, fibro purulent and organized. ☒

C. C-Empyema has no stages. ☐

D. Early stage and late stage. ☐

1/1 * Which of the following types .25 ✓
of lung cancer causes
?gynaecomastia

A. Small cells lung cancer. ☐

B. Adenocarcinoma. ☐

C. Squamous cell carcinoma. ☐

✓ D. Large cell carcinoma. ☒

لم يتم إنشاء هذا المحتوى ولا اعتماده من قبل Google. الإبلاغ عن إساءة
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1/1* What is the most common .1 ✓
frequent cause of exudative
?pleural effusion

A. Tuberculosis. ☐

B. Parapneumonic pleural effusion. ☐

✓ C. Malignancy. ☒

D. Lymphatic obstruction. ☐

أخرى: ☐

1/1* Which of the following is the .2 ✓
most common cause of high
protein content pleural effusion
?with normal serum protein

✓ A. TB. ☒

B. Liver cirrhosis. ☐

C. Urinothorax. ☐

D. Heart failure. ☐

1/1* What is the most common .3 ✓
cause for increase amylase in
?pleural fluid



A. Esophageal rupture. ☒

B. Renal failure. ☐

C. Tuberculosis. ☐

D. Chylothorax. ☐

1/1* Whisper pectrology is .4 ✓
positive in which of the following
?diseases

A. Pulmonary fibrosis. ☐



B. Pneumonia. ☒

C. Chronic bronchitis. ☐

D. Pleural effusion. ☐

1/1* Which of the following .5 ✓
diseases is associated with
?orthopnea



1/1*

Which of the following .5
diseases is associated with
?orthopnea



A. Pneumonia. ☐

B. Pneumothorax. ☐



C. Left ventricular failure. ☒

D. Right ventricular failure. ☐

1/1*

Which of the following .6
diseases causes absent litten
?signs



A. Bronchial asthma. ☐



B. Pleural effusion. ☒

C. Pneumonia. ☐

D. Acute exacerbation of COOD. ☐

1/1*

You are asked to interpret an .7
arterial blood gas of a 76-year-
old patient who admitted to
accident and emergency with an
acute onset of breathlessness



1/1* You are asked to interpret an .7 ✓
arterial blood gas of a 76-year-old patient who admitted to accident and emergency with an acute onset of breathlessness and low oxygen saturations. The test was taken on room air and read as follows : pH 7.37, PO₂ 58 mmHg, PCO₂ 31 mmHg, HCO₃⁻ 24, SO₂ 89%. Choose the most likely clinical interpretation from these arterial blood gas results

A- Compensated respiratory acidosis. ☐

✓ B- Type 1 respiratory failure. ☒

C- Uncompensated respiratory alkalosis. ☐

D- Type 2 respiratory failure. ☐

1/1* Which one of the following .8 ✓
conditions causes respiratory failure primarily by hypoventilation

A. Pneumonia ☐

✓ B. Myasthenia gravis ☒



C. Pulmonary fibrosis ☐

D. Pulmonary embolism ☐

1/1*

Which of the following .9 ✓
causes of respiratory failure is
recognized to have the highest
?mortality rate

A. Chronic obstructive pulmonary
disease (COPD) ☐

B. Asthma ☐

✓ C. Acute respiratory distress
syndrome (ARDS) ☒

D. Obesity ☐

1/1 * ?What does hypoxia mean .10 ✓

A. Increased hemoglobin in the blood. ☐

✓ B. Decreased oxygen reaching
bodily tissues. ☒

C. Decreased hemoglobin in the blood. ☐

D. Increased oxygen reaching bodily
tissues. ☐



1/1* Which one of the following .11 ✓
is NOT a symptom of
?hypoxemia

A. Cyanosis ☐

B. Headache. ☐

C. Irregular heartbeats. ☐



D. Increased energy. ☒

1/1* Loss of sense of right and .12 ✓
:wrong occur when

A. Partial pressure of oxygen (PaO₂)
above 50 mmHg. ☐

✓ B. Partial pressure of oxygen
(PaO₂) is below 35 mmHg. ☒

C. Partial pressure of oxygen (PaO₂)
35-50 mmHg. ☐

D. Partial pressure of oxygen (PaO₂)
above 65 mmHg. ☐

1/1* The BCG vaccine is .13 ✓
administered for immunity



1/1*

The BCG vaccine is .13 ✓
administered for immunity
against

A. Malaria. ☐



B. Tuberculosis ☒

C. Jaundice. ☐

D. Hepatitis. ☐

1/1*

Which one of the following .14 ✓
is a first line drug for TB
?treatment



A. Pyrazinamide. ☒

B. Paraaminosalicylic acid (PASA). ☐

C. Kanamycin ☐

D. Cycloserine. ☐

1/1*

The gold standard test for .15 ✓
identification and sensitivity of
:TB bacilli is

1/1*

The gold standard test for .15
identification and sensitivity of
:TB bacilli is



A. Specimens ☐

B. Direct microscopy of specimen ☐



C. Culture ☒

D. None of these ☐

1/1*

In mediastinal syndrome .16
hoarseness of voice is due to
:compression on



A. Sympathetic trunk. ☐



B. Left recurrent laryngeal nerve. ☒

C. Esophagus ☐

D. Trachea. ☐

1/1*

A 20-year-old, female, .17
presented with paroxysmal
attacks of dyspnea and chest



1/1*

A 20-year-old, female, .17 presented with paroxysmal attacks of dyspnea and chest wheezes. Her chest X-ray detected no abnormalities but her spirometry tests revealed normal FVC, low FEV1 and FEV1/FVC. The most probable diagnosis is



A. Restrictive lung disease.



B. Obstructive lung disease.



C. Chest wall disease.



D. Vascular lung disease.



1/1*

A 15-year-old, male, .18 basketball player presented at emergency department by severe stitching left sided chest pain and dyspnea. There was no available chest X-ray. Ultrasound was done and revealed absent left lung sliding with horizontal reverberation artifacts. The most probable diagnosis is



1/1*

A 15-year-old, male, .18 ✓
basketball player presented at
emergency department by
severe stitching left sided chest
pain and dyspnea. There was no
available chest X-ray. Ultrasound
was done and revealed absent
left lung sliding with horizontal
reverberation artifacts. The most
:probable diagnosis is

A. Pleural effusion. ☐



B. Pneumothorax. ☒

C. Pulmonary embolism. ☐

D. Pneumonia ☐

1/1*

Rigid bronchoscopy is .19 ✓
preferentially indicated for

A. Persistent pneumonia ☐



B. Extracting foreign bodies ☒

C. Atelectasis ☐

D. Persistent wheeze ☐



1/1*

A 10-month-old infant has poor weight gain, a persistent cough, and a history of several bouts of pneumonitis. The mother describes the child as having very large, foul-smelling stools for months. Which of the following diagnostic maneuvers is likely to result in the correct diagnosis of this child



A. CT of the chest ☐

B. Serum immunoglobulin's ☐

C. Inspiratory and expiratory chest x-ray ☐



D. Sweat chloride test. ☒

1/1*

Which of the following in Pediatric airway is different from adult



A. Large tongue. ☐



B. Short epiglottis. ☒

C. Narrowest part is glottis ☐

D. Larynx in lower position. ☐



1/1*

Bilateral pneumothorax .22 ✓
.....managed by

A. Thoracentesis ☐

B. Observation ☐



C. Thoracotomy exploration. ☒

D. Needle aspiration. ☐

1/1*

Lung abscess is diagnosed .23 ✓
by

A. Thoracoscopy. ☐

B. Bronchoscope. ☐

C. Mediastinoscopy. ☐



D. Computed tomography. ☒

1/1*

Blood pressure in the .24 ✓
pulmonary artery is



1/1*

Blood pressure in the .24 ✓
_____ pulmonary artery is

- A. Same as that in the aorta. ☐
- B. Less than that in venae cavae. ☐
- C. More than that in the carotid. ☐
- ✓ D. More than that in the pulmonary vein. ☒

1/1* Which of the following tests .25 ✓
?is least relevant in sarcoidosis

- A. 24 hour urinary excretion of calcium ☐
- ✓ B. 24 hour proteinuria ☒
- C. Slit- lamp and fundus examination ☐
- D. Serum angiotensin converting enzyme ☐

لم يتم إنشاء هذا المحتوى ولا اعتماده من قبل Google. الإبلاغ عن إساءة الاستخدام - شروط الخدمة - سياسة الخصوصية

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